PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Experiences of older immigrants living with dementia and their carers: a systematic review and meta-synthesis
AUTHORS	Chejor, Pelden; Laging, Bridget; Whitehead, Lisa; Porock, Davina

VERSION 1 – REVIEW

REVIEWER	1: Lazzari, Carlo Fieldhead Hospital
REVIEW RETURNED	08-Jan-2022

GENERAL COMMENTS	Esteemed authors I appreciate and enjoyed reding the article. You demonstrate a profound knowledge of the methods applied as meta-synthesis is not an easy method to use My only suggestion that after 'search search strategy' I would add the table of title extracted for each of the Boolean connectives used, providing the date when the extraction occurred, and dividing the keywords before giving the total of titles, i.e.. Table of extracted titles ========== #1. 'older OR elder* OR senior* OR geriatric OR adult* OR aged': (@ of titles extracted) #2 . 'immigrant* OR emigrant* OR migrant* OR foreign-born OR overseas-born OR non-English-speaking': (@ of titles extracted) #3 'dementia OR Alzheimer's OR "cognitive impairment" OR "cognitive disorders" OR "memory loss" OR "memory disorders": ('number of titles) #1 AND #2 AND #3: (number of final titles extracted) ========== I hope this can help. Good luck with your research. Reviewer No1

REVIEWER	Laver, Kate Flinders University
REVIEW RETURNED	09-Jan-2022

GENERAL COMMENTS	This is a well conducted review. Some minor points for consideration below. The main thing missing is the 'so what?' factor. It would be helpful for the authors to be a bit bolder in their

	recommendations given they have included 18 studies – this should be sufficient to make some suggestions. Abstract: include study design type. More detail on synthesis method. Abstract: reorganize results and conclusion so most findings are in results. Please eliminate abbreviations as they are hard to follow: OIWD, HCW, FCG and not commonly used The authors identified 18 studies reported in 20 papers. I would suggest reporting this as 18 studies (and link multiple papers to same study where applicable) and this is done well in Table 1. Please check as in the study description the numbers don't add up, eg methods for 21 studies (13 + 5 + 2 + 1). A little more detail on the thematic synthesis is required. What exact process took place? Describe in replicable manner. Results: Please describe the populations in the included studies. How were they recruited? Were they particular immigrant groups? Figure 2 has some formatting issues The findings amongst immigrants are not that different to findings amongst a broader population. Suggest discussion re this in Discussion.

VERSION 1 – AUTHOR RESPONSE

Reviewer: 1
Mr. Carlo Lazzari, Fieldhead Hospital
Comments to the Author:
1. Esteemed authors I appreciate and enjoyed reading the article. You demonstrate a profound knowledge of the methods applied as meta-synthesis is not an easy method to use

Response: Thank you for your comments.

2. My only suggestion that after 'search strategy' I would add the table of title extracted for each of the Boolean connectives used, providing the date when the extraction occurred, and dividing the keywords before giving the total of titles, i.e..

Table of extracted titles

==========
#1. 'older OR elder* OR senior* OR geriatric OR adult* OR aged': (@ of titles extracted)

#2 . 'immigrant* OR emigrant* OR migrant* OR foreign-born OR overseas-born OR non-English-speaking': (@ of titles extracted)

#3 'dementia OR Alzheimer's OR "cognitive impairment" OR
"cognitive disorders" OR "memory loss" OR "memory disorders": ('number of titles)

#1 AND #2 AND #3: (number of final titles extracted)
==========
I hope this can help.
Good luck with your research.

Response: Thank you for your comments. We have included a table of extracted titles as Table 1 in the manuscript as shown below.

Table 1 Titles extracted from each database using the search query

	Search Query	Database CINAHL	MEDLINE	PsycINFO	EMBASE	WOS	PubMed	CL
S1	older OR elder* OR senior* OR geriatric OR adult* OR aged	2,569,203	8,676,527	2,369,350	11,301,861	6,361,200	8,929,504	954,140
S2	immigrant* OR emigrant* OR migrant* OR foreign-born OR overseas-born OR non-English-speaking	52,194	60,299	41,150	6,80,067	153,622	60,018	1,588
S3	dementia OR Alzheimer's OR "cognitive impairment" OR "cognitive disorders" OR "memory loss" OR "memory disorders"	165,349	362,851	157,024	455,278	377,185	356,477	32,456
S4	#1 AND #2 AND #3	2,417	530	284	432	402	367	46
S5	S4 narrowed by language (English), peer-reviewed and publication dates (Jan 2000 – April 2021)	2,144	452	217	398	333	271	42

WOS: Web of Science; CL: Cochrane Library
search date: 05/04/2021

Reviewer: 2
Dr. Kate Laver, Flinders University

Comments to the Author:
1. This is a well conducted review. Some minor points for consideration below. The main thing missing is the 'so what?' factor. It would be helpful for the authors to be a bit bolder in their recommendations given they have included 18 studies – this should be sufficient to make some suggestions.

Response: Thank you for your comments. We have revised the recommendations to reflect your encouragement.

Many countries, including Australia, welcome migrants and are working towards the goal of multiculturalism. For this to hold true for all citizens and residents, including well into late life, it means that aged care and dementia services must go the extra mile to give those who need to migrate and chose to stay in the new country, the care they need at this vulnerable time in their lives. From an implementation angle, we suggest raising public awareness campaigns using appropriate and culturally sensitive language and increasing dementia training for carers, as also recommended by Alzheimer's Disease International, to reduce dementia stigma.

2. Abstract: include study design type. More detail on synthesis method.

Response: Thank you for your comments. We have included the study design type in the abstract as follows:

Design: This was a systematic review and meta-synthesis of qualitative studies.

3. Abstract: reorganize results and conclusion so most findings are in results.

Response: Thank you for your comments. We have reorganized results and conclusion to ensure most findings are in results, as follows:

Results The results of this meta-synthesis were reported according to the Preferred Reporting Items for Systematic Reviews and Meta-Analysis statement and Enhancing transparency in reporting the synthesis of qualitative research statement. A total of 3857 studies were returned from the database search. Eighteen studies were included for meta-synthesis. Five synthesised findings were identified: living with dementia and caregiving; family relationships; barriers to dementia care services; stigma and discrimination; and legal and financial issues. The experiences of living with dementia and caregiving presented multiple challenges for older immigrants living with dementia and their families. However, there seems to be very little difference between the experiences of those who have migrated to a new country and those who were born and aged in the same country, but the ability to access and utilise the available services is different.

Conclusion A lack of culturally appropriate dementia services, language barriers and dementia stigma can impede access to dementia care for older immigrants. Strategies to mitigate these barriers is urgently needed to ensure people from culturally and linguistically diverse backgrounds with dementia and their families have the information, education, and support to access dementia services, in addition to research that explores the experiences of culturally and linguistically diverse population.

4. Please eliminate abbreviations as they are hard to follow: OIWD, HCW, FCG and not commonly used

Response: Thank you for your comments. We have replaced all these abbreviations throughout the manuscript (OIWD: older immigrants living with dementia; HCW: healthcare workers; FCG: family caregivers).

5. The authors identified 18 studies reported in 20 papers. I would suggest reporting this as 18 studies (and link multiple papers to same study where applicable) and this is done well in Table 1.

Response: Thank you for your comments. We have reported as 18 studies, as follows:

PC independently appraised all 18 articles, where two reviewers (LW and BL) peer-reviewed 10 articles each.

After a full-text assessment of 263 articles, 18 studies were included in the meta-synthesis.

6. Please check as in the study description the numbers don't add up, eg methods for 21 studies (13 + 5 + 2 + 1).

Response: Thank you for your comments. We have corrected the number in the study descriptions of included studies (13 + 4 + 2 + 1).

Individual interviews were used in 13 studies,[31 32 35 36 39 40 43-46 48 49 51] with four studies using focus group interviews,[37 38 47 50] two studies using both individual interviews and focus groups,[33 34] and one study used mixture of individual, dyad, and focus group interviews.[16]

7. A little more detail on the thematic synthesis is required. What exact process took place? Describe in replicable manner.

Response: Thank you for your comments. We have added detail processes on thematic synthesis. It was an inductive approach as the themes were generated based on the data. First, PC carefully read and re-read each included studies to get familiarised with the included studies. Second, all the included studies were coded line-by-line. All the coded texts were checked to ensure consistency of interpretations and when necessary new codes were created. Third, similar codes were grouped and categorised as subthemes. Subthemes across articles were compared and matching subthemes from different articles were grouped together. PC re-assessed and reorganised subthemes and themes throughout the process. Finally, similar subthemes were then grouped into a larger theme and interpreted using the SEM framework. Findings, subthemes, and themes were examined by all other reviewers to ensure rigour.

8. Results: Please describe the populations in the included studies. How were they recruited? Were they particular immigrant groups?

Response: Thank you for your comments. We have described the populations in the included studies, as follows:

Participants were from Poland, Croatia, Finland, China, Korea, Vietnam, Pakistan, India, Bangladesh, Sri Lanka, Iran, Moroccan, Turkey, Lebanon, Africa, and South America. Most of the participants were females. Participants were recruited via community organisations (dementia centres, mental health teams, nursing homes, memory clinics, religious centres) and key figures in the community[31-34 39 43 44 49-51] followed by via community organisations alone.[16 37 38 40 45 47 48] Purposive sampling[16 31 35 37 38 48 49 51] was commonly used to recruit participants, with snowball[39 43 44] and convenience[40] sampling in few studies.

9. Figure 2 has some formatting issues

Response: Thank you for your comments. We have removed the colour of Figure 2 which has led to formatting issues, as shown below.

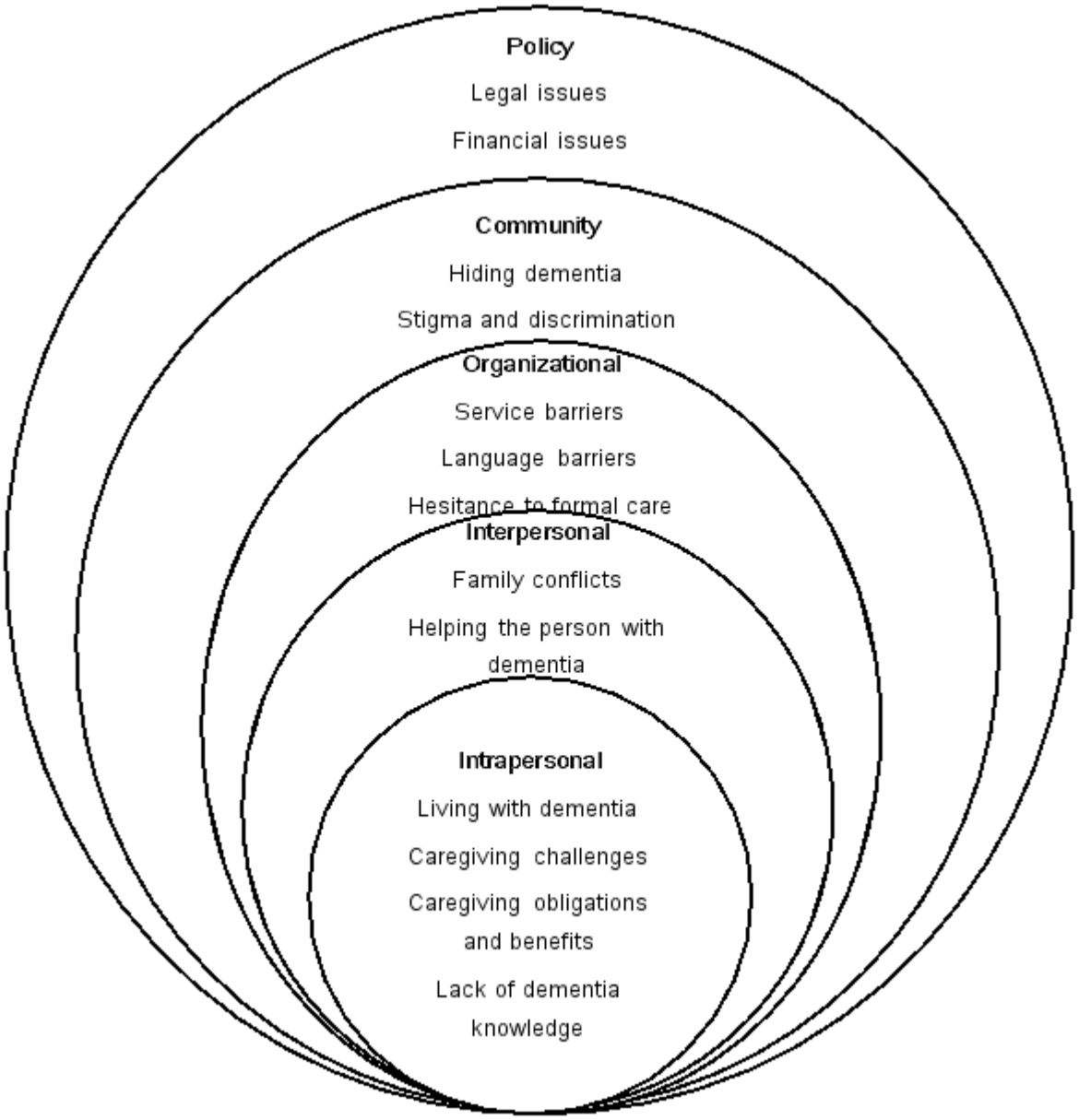


Figure 2 Themes and subthemes arranged in socio-ecological model framework

10. The findings amongst immigrants are not that different to findings amongst a broader population. Suggest discussion re this in Discussion.

Response: Thank you for this comment. We have no discussed this under the Discussion.
The distress, stigma and overwhelming human experience of dementia is shared by all those that live with the condition and the people who love and care for them. The fact that there seems to be very little difference between the experiences of those who have migrated to a new country and those who were born and aged in the same country, demonstrates the inequity that simply coming from a CALD

community brings. The experience is the same, the needs are the same, but the ability to access and utilise the available services is different. This means that more attention must be given to ensuring people from CALD backgrounds with dementia and their families have the information, education, and support to access services for help and respite.